



Tel. No. 929-75-39

EPIDEMIOLOGICAL QUESTIONNAIRE (Neo-natal Tetanus)

Health Center:		District:	
Investigator:		Date Investigated:	
1. PATIENT INFORMATION			
From Endorsement? ☐ Yes ☐ No		EPI ID:	
Last Name:		First Name:	Middle Name:
			Suffix:
Date of Birth: ____/____/____ MM DD YYYY	Age: ☐ days ☐ weeks	Current Address: Length of residency:	
Sex at birth: ☐ Male ☐ Female		Permanent Address:	
Religion:		Length of residency:	
Date of Report:		Date of Investigation:	
2. MOTHER'S INFORMATION			
Prenatal Care No. of total pregnancies: Live births: ____ Living children: ____ How many prenatal care visits did the mother make to a health facility during her pregnancy? When was the first prenatal visit? ____/____/____ Is the prenatal care history reported by: ☐ Card ☐ Recall ☐ Both ☐ Unknown State reason for no or late prenatal care: _____ _____ _____	Immunization Status How many doses of TT has the mother received? ____doses ____unknown Date last dose given: ____/____/____ If she received 2 doses, were they given during pregnancy? ☐ Yes ☐ No ☐ Unknown Is the immunization status reported by: ☐ Card ☐ Recall ☐ Both ☐ Unknown If the mother DID NOT receive Tdap vaccine, state the reason(s): _____ _____	If she has a card, copy the dates of all TT immunization recorded on the card: TT1: ____/____/____/ TT2: ____/____/____/ TT3: ____/____/____/ TT4: ____/____/____/ TT5: ____/____/____/ Is the child protected at birth? ☐ Yes ☐ No ☐ Unknown	
2.1 CLINICAL INFORMATION			
In the first 2 days of life did the baby has normal suck and cry?		After 2 days of life, was the baby unable to suck and cry normally?	
☐ Yes ☐ No ☐ Unknown		☐ Yes ☐ No ☐ Unknown	
After 2 days of life, did the baby have body stiffness / rigidity?		Was the umbilical stump infected? (foul smelling / pus)	
☐ Yes ☐ No ☐ Unknown		☐ Yes ☐ No ☐ Unknown	
Did the baby have spasm / convulsion?		Did the baby have locked jaws (opisthotonos)?	
☐ Yes ☐ No ☐ Unknown		☐ Yes ☐ No ☐ Unknown	



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Complications:

- ☐ Asphyxia (Laryngeal/diaphragm spasms)
☐ Sepsis (Umbilical infection)
☐ Aspiration pneumonia
☐ Pulmonary hemorrhage
☐ Long term neurodevelopmental disabilities

Others: _____

Consultation:

- ☐ Yes ☐ No
☐ Health Center: _____ Date: __/__/____
☐ Private Clinic: _____ Date: __/__/____
☐ Tele-med: _____ Date: __/__/____
☐ Hospital: _____ Date: __/__/____

Diagnosis:

Admission:

- ☐ Yes ☐ No
☐ Government Hospital: _____ Date Admitted: __/__/____ Date Discharged: __/__/____
☐ Private Hospital: _____ Date Admitted: __/__/____ Date Discharged: __/__/____

3. DELIVERY PRACTICES

Place of delivery ☐ Home ☐ Hospital / Lying- in / Clinic ☐ Other; specify _____

If born in a hospital / lying-in / clinic, give name and address of the hospital / lying -in ; _____

Who attended the delivery? ☐ Physician ☐ Nurse ☐ Midwife ☐ Hilot ☐ Unknown ☐ Other; specify _____

Cord was cut using: ☒ Scissors ☐ Blade ☐ Bamboo ☐ Unknown ☐ Other; specify _____

If Hilot, was he / she trained? ☐ Yes ☐ No ☐ Unknown

Stumo treated (dressed) with: ☐ Alcohol ☐ Povidone-iodine ☐ Unknown ☐ Other; specify _____

4.0 Case Classification

Suspect Case

Any neonate who would suck and cry normally during the first two days of life and develop tetanus-like illness or death between 3 and 28 days of age and not investigated

Clinically-Confirmed Case

- Any suspected case found to have all three of the following:
- normal ability to suck and cry during the first 2 days of life *AND*
 - could not suck normally between 3 and 28 days of age *AND*
 - developed muscle stiffness and/ or spasms which may include jerking, trismus, clenched fists or feet, continuously pursed lips, and/or curved back (opisthotonus)

Note: Neonatal tetanus case classification is based solely on clinical criteria. Any neonatal death occurring in babies 3-28 days old with no apparent cause should be suspected as NT and evaluated according to the above criteria. In calculating age, the day of birth is considered the first day of life (i.e.,the baby is 1 day old on the day he/she was born.

4.1 Outcome

- ☐ Alive
☐ Died, Date (MM/DD/YYYY): _____

4.3. Health Status

- ☐ Still sick
☐ Recovered, Date (MM/DD/YYYY): _____



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5. INTERVENTIONS AND CONTROL MEASURE		
ACTIONS	DATE	REMARKS
☐ Case Management		
☐ Referral to Hospital		
☐ Immediate Hospital Admission		
☐ ICU / critical care management		
☐ Others:		
☐ Field Investigation		
☐ Exposure source identified		
☐ Contact tracing initiated		
☐ Cluster detection		
☐ Others:		
☐ Laboratory Confirmation		
☐ Sample collected		
☐ Program Mgmt. & Counter Measures		
☐ Strengthening Immunization program		
☐ Promotion of skilled Birth Attendance		
☐ Health education and Community Engagement		
☐ Logistic and Supply Management		
☐ Others:		
☐ Health Education		
☐ Recommend Maternal Immunization		
☐ Promote Clean and Safe Delivery practice		
☐ Proper umbilical cord care		
☐ Avoid Harmful Traditional Practices		
☐ Others:		
☐ Response Coordination Mechanism		
☐ Coordination with DRU		
☐ Immediate Public Health Response		
☐ Immediate Case Notification		
☐ Others:		
☐ Risk Assessment		
☐ Maternal Risk Factor		
☐ Newborn Risk Factor		
☐ Health System Factors		
☐ Community / Population Risk Indicators		
☐ Others:		



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7. SURVEY AND OBSERVATION	
Socio-demographic Profile <i>(based on the total monthly income of household)</i> :	
· Poor (<P12,030) · Low (>P12,030 to <P24,060) · Lower Middle (> P24,060 to <P48,120) · Upper Middle (>P84,210 to <P144,360) · Upper Income (>P144,360 to <P240,060) · Rich (>P240,060)	
Type of house residence · Apartment · Gated Subdivision · Condominium · Densely-connected houses	Overall condition of the house · Excellent · Good · Fair · Poor

Interviewee: ☐ Parent ☐ Guardian ☐ Relative other than parents ☐ Patient himself/herself ☐ Others:
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